Discharge, Transfer and Death Documentation

The documents a patient leaves with — and the five different routes that each need their own set.

Module	08 of 12
Study time	100 minutes
Audience	Clinical and nursing staff, medical records staff, billing, mortuary and administration
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition AAC chapter on discharge; COP chapter on end-of-life care; Registration of Births and Deaths Act 1969; MCCD requirements

The discharge summary is the only document in the file that the patient actually takes away and the only one the next hospital will certainly read. It is also the document most often written in a hurry by the most junior person available. This module covers the summary's required contents, the four other ways a patient can leave and the different document set each needs, and the documentation of death from certification through to body handover.

LEARNING OUTCOMES — by the end of this module you will be able to:

- Write a discharge summary containing every required element, in language the patient can use
- Distinguish the five exit routes and produce the correct document set for each
- Document a discharge against medical advice and an absconding patient
- Complete a transfer-out summary that allows the receiving hospital to continue care
- Document a death, complete the cause-of-death structure correctly, and record the body handover
- Identify deaths requiring medico-legal referral and document the referral

8.1 Why the discharge summary carries disproportionate weight

Three readers depend on it almost entirely. The patient, who will follow its instructions or not understand them. The next clinician — a general practitioner, a hospital in another city, an emergency department at 2 a.m. — who will have nothing else. And the payer, who will decide the claim largely on what it says. A thin summary therefore produces clinical risk, a confused patient and a rejected claim simultaneously.

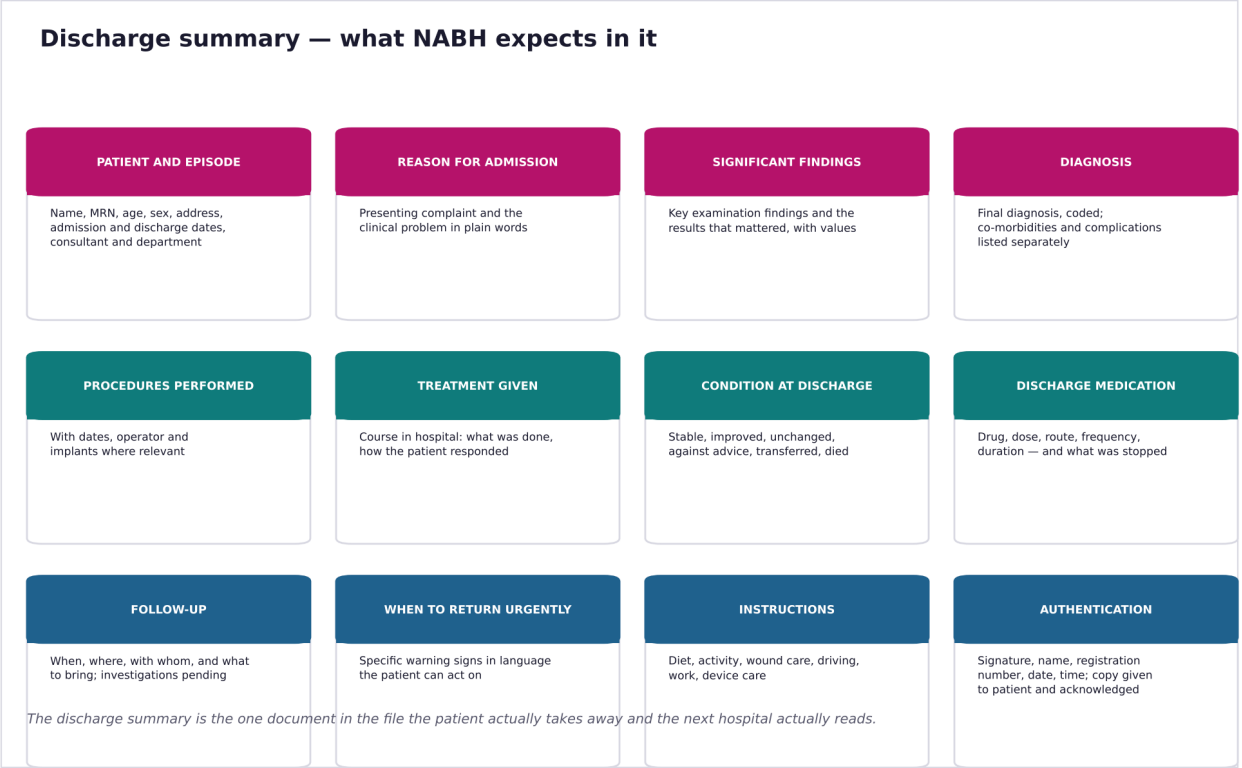


Figure 8.1 — Required contents of a discharge summary.

8.2 Writing each section properly

Section	What good looks like	What is usually written
Reason for admission	The clinical problem stated in plain words with its duration	A one-word symptom
Significant findings	The examination findings and the results that actually changed management, with values and dates	‘Investigations done’ with reports attached and nothing interpreted
Diagnosis	Final diagnosis with the site, type, severity and stage where applicable; co-morbidities and complications listed separately	The admitting provisional diagnosis, unchanged, or a symptom
Procedures	Name, date, operator, findings in brief, implants with details	‘Surgery done’
Course in hospital	What was done, how the patient responded, what complications occurred and how they were managed	‘Patient improved with treatment’
Condition at discharge	An explicit statement: stable, improved, unchanged, against advice, transferred, died, with functional status	Omitted entirely
Discharge medication	Drug, strength, dose, route, frequency, duration, and explicitly what has been stopped and what is new	A list of brand names with no duration and no indication of changes
Follow-up	When, where, with whom, what to bring, and which results are still pending and who will chase them	‘Review after one week’
Return urgently if	Specific warning signs in language the patient can act on	‘Report if any problem’
Instructions	Diet, activity, wound care, driving, return to work, device care, what the patient may and may not do	‘Take rest’

KEY POINT
Write the ‘return urgently if’ section for the patient, not the doctor. ‘Come back at once if the wound becomes red, hot, or starts leaking, if you develop fever above 100 degrees, or if the pain gets worse instead of better’ is usable. ‘Report if any complication’ is not.

The summary must be authenticated by a clinician — signature, name, designation and registration number — and a copy must be given to the patient with evidence that it was received. The acknowledgement matters: the commonest counter-allegation in a late-presentation case is that no summary was provided.

8.3 Pending results at discharge

Patients are frequently discharged with investigations outstanding — cultures, histopathology, specialist reports. The summary must name each pending investigation, say who is responsible for reviewing it, and say how the patient will be told the result. A summary that is silent on a pending histopathology report is the first link in the chain that ends with an unreported malignancy.

8.4 The five exit routes

Five ways a patient leaves — five different document sets

Route	Core document	Additional documents required
Planned discharge	Discharge summary	Prescription, follow-up appointment, patient copy acknowledged, final bill
Discharge against medical advice	Discharge summary + DAMA form	Clinical note of the conversation, risks explained, witness signature, consultant informed
Transfer to another hospital	Transfer summary	Reason for transfer, receiving hospital and doctor accepting, mode of transport, condition at transfer, copies of investigations sent
Absconding	Incident note	Time last seen, search conducted, police and relatives informed and when, consultant and administration notified
Death	Death summary	Time and cause of death, who certified, MCCD / death certificate, body handover record, medico-legal referral where applicable

Using the planned-discharge document set for any of the other four is the single commonest medico-legal exposure at exit.

Figure 8.2 — Five ways a patient leaves, and the document set each requires.

Using the planned-discharge document set for any of the other four is the single commonest medico-legal exposure at exit. Each route is considered below.

8.5 Discharge against medical advice

The DAMA form is the weakest part of the documentation and is usually treated as the whole of it. What protects the hospital is the clinical note of the conversation. Module 6 sets out the sequence; the points that bear repeating here are that the patient must still receive a discharge summary, a prescription and specific return advice, and that the consultant and duty officer must be informed and recorded.

Record the patient’s own reason in their words. A large proportion of DAMA departures in Indian hospitals are driven by cost, by transport, by a family member needing care at home, or by a misunderstanding about the expected length of stay. Documented, these are frequently solvable, and the record then shows a hospital that

tried.

8.6 Transfer to another hospital

A transfer-out summary is not a shortened discharge summary. Its purpose is to let the receiving team continue care without restarting, so it must carry the clinical reasoning and not only the conclusions.

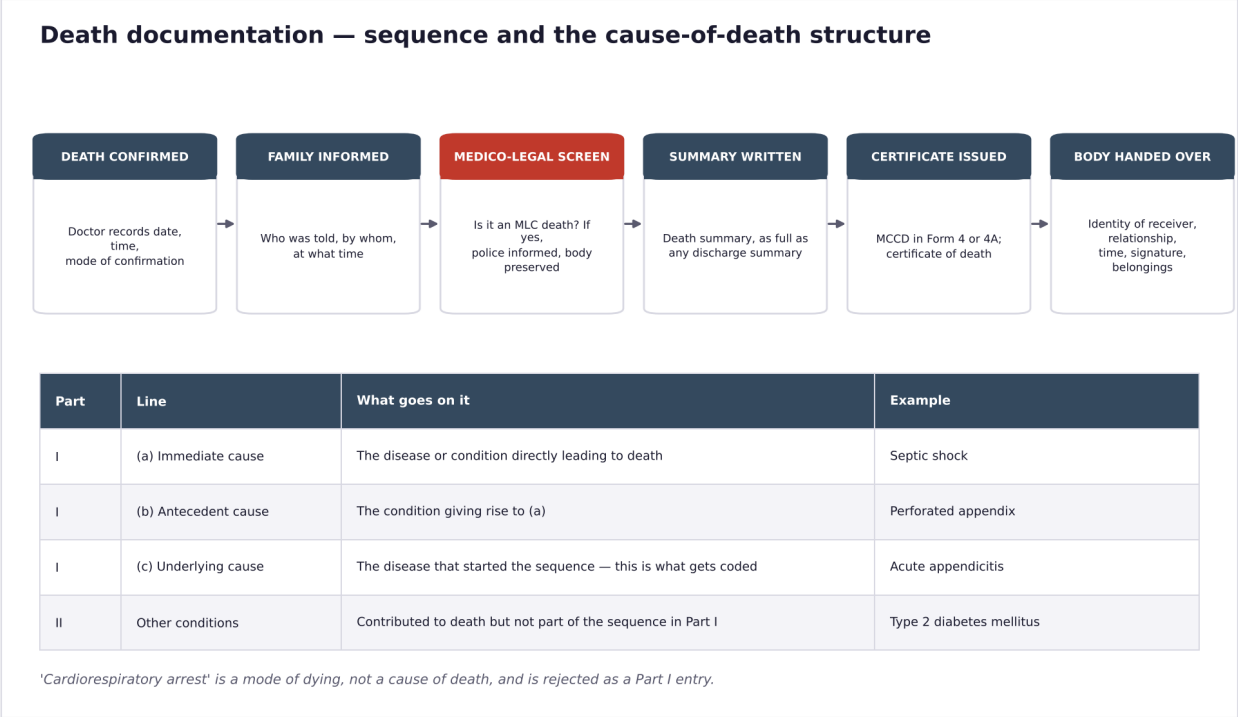
- The reason for transfer, stated honestly — a service the hospital cannot provide, a bed, a patient request, an insurance requirement
- The name of the receiving hospital and of the doctor who accepted the patient, with the time of acceptance
- The clinical condition at the time of transfer, with the last set of observations and their time
- Treatment given, with the last doses and times of time-critical drugs such as antibiotics, anticoagulants and insulin
- Lines, drains, tubes and devices in situ, with insertion dates
- Investigations done, with copies of the reports and imaging transferred with the patient
- Investigations outstanding and who will follow them up
- Mode of transport, the level of escort, and the equipment and drugs sent with the patient
- Consent for the transfer, and what the patient or family was told about the reason and the risks

> **CAUTION**
> Where a patient is transferred because the hospital cannot provide a required service, the record must show when that was recognised and what happened in between. A gap between recognising the need and arranging the transfer is the point at which these cases turn.

8.7 Absconding

An absconding patient is an incident and generates an incident record, not a discharge summary. The record must show the time the patient was last seen, the time the absence was noticed, the search conducted, who was informed and when — relatives, consultant, administration and, where the patient is a minor, a person lacking capacity, a medico-legal case or a person at risk, the police. The patient’s belongings and any devices still in situ should also be recorded.

8.8 Documenting a death



Part	Line	What goes on it	Example
I	(a) Immediate cause	The disease or condition directly leading to death	Septic shock
I	(b) Antecedent cause	The condition giving rise to (a)	Perforated appendix
I	(c) Underlying cause	The disease that started the sequence — this is what gets coded	Acute appendicitis
II	Other conditions	Contributed to death but not part of the sequence in Part I	Type 2 diabetes mellitus

'Cardiorespiratory arrest' is a mode of dying, not a cause of death, and is rejected as a Part I entry.

Figure 8.3 — The death documentation sequence and the cause-of-death structure.

The sequence begins with the confirmation of death, recorded with the date, the time and the method of confirmation by the doctor who confirmed it. The clinical record must contain a final entry describing the terminal events, the resuscitation attempted or the documented decision not to attempt it, and who was present.

The death summary

A death summary is written to the same standard as a discharge summary: reason for admission, significant findings, diagnoses, procedures, course in hospital, the terminal events, the cause of death, and the communication with the family. Hospitals frequently produce a two-line death note, which is precisely the file that will be scrutinised most closely.

Certification of cause of death

The Medical Certification of Cause of Death is a statutory document completed in Form 4 for institutional deaths, or Form 4A for non-institutional deaths. Part I records the sequence leading directly to death, from the immediate cause at (a) to the underlying cause at the lowest completed line. The underlying cause — the disease that started the sequence — is the one that is coded and the one that appears in national mortality statistics. Part II records other significant conditions that contributed but were not part of the Part I sequence.

Common error	Why it is wrong	Correct entry
‘Cardiorespiratory arrest’ written at Part I (a)	It is the mode of dying, not a cause; every death ends in it	The actual immediate cause, for example ‘septic shock’ or ‘massive pulmonary embolism’
Only one line completed, with the immediate cause	The underlying cause, which is what gets coded, is missing	Complete the sequence downward to the condition that started it
‘Old age’ or ‘natural causes’	Not a certifiable cause in most circumstances	The specific disease that caused death
Mechanism without antecedent, such as ‘renal failure’ alone	Renal failure is a consequence of something	‘Acute kidney injury’ at (a), due to ‘septic shock’ at (b), due to ‘perforated diverticulitis’ at (c)

Common error	Why it is wrong	Correct entry
Abbreviations and unexplained acronyms	Cannot be coded reliably	Write conditions in full
Multiple unrelated conditions at Part I	Part I must be a single causal sequence	Keep the sequence in Part I; place other contributors in Part II

KEY POINT
The person completing the certificate should be a doctor who attended the patient and who can state the cause from knowledge of the case. Certifying a death the certifier knows nothing about, from the file alone, is both poor practice and the source of most uncodeable certificates.

8.9 Medico-legal deaths

Certain deaths must not be certified in the ordinary way and must be referred for medico-legal investigation. The screening decision must be documented in every death, not only in the obvious ones, and the hospital should have a standing list.

- Any death from injury, accident, burns, poisoning or suspected poisoning
- Death in suspicious circumstances or where foul play is alleged
- Death following an alleged assault, including domestic violence
- Suspected suicide
- Death during or shortly after a procedure or anaesthesia, where the death was unexpected
- Death of a woman within forty-two days of termination of pregnancy or delivery, where the circumstances warrant it
- Death within twenty-four hours of admission where the cause is not established
- Death of a person in custody
- Brought-dead cases where the cause is unknown

In these cases the record must show the referral, the time the police were informed, the preservation of the body and of any material evidence, and that no certificate was issued ahead of the statutory process.

8.10 Body handover

The handover record is the hospital's last document and it is frequently the least controlled. It must record the identity of the person receiving the body with proof of identity, their relationship to the deceased, the date and time, an inventory of belongings and valuables handed over, the condition of the body, and the signatures of the receiver and the releasing officer. Where the body is unclaimed or unidentified, the process follows the statutory route and must be documented in full.

8.11 End-of-life documentation

Where a decision is made to limit or withdraw treatment, the documentation standard is higher than for any decision to give treatment. The record must show the clinical basis, the discussion with the patient where possible and with the family, any advance directive, who participated in the decision, what was decided and its scope, and the plan for symptom control. A 'do not resuscitate' instruction recorded as three letters on a chart with no supporting discussion is indefensible.

8.12 The discharge process and who does what

Discharge summaries are late because the discharge process is undefined, not because clinicians are unwilling. A defined process with named steps produces summaries at discharge; an undefined one produces them when the records department chases.

Step	Owner	Timing
Decision to discharge made and recorded	Treating consultant or the team on their behalf	On the round, with the criteria met recorded
Discharge summary drafted	Treating team	Same day as the decision, before the patient leaves
Medication reconciliation completed	Prescriber, with pharmacy where available	Before the prescription is written
Summary authenticated	Consultant or delegated clinician	Before handover to the patient
Patient counselled	Nursing, with the clinician for clinical content	At handover, with counselling recorded
Copy handed over and acknowledged	Nursing or records staff	At the point the patient leaves
File completed and sent to the records department	Ward	Within twenty-four hours

Planned discharge — identifying the likely discharge date at admission and working towards it — is the single intervention that most improves summary timeliness, because the summary can be drafted across the admission rather than written in the last ten minutes.

8.13 Writing for the reader who will actually use it

A discharge summary written in abbreviations for another doctor fails the patient, and a summary written only in lay terms fails the next clinician. The practical answer is a summary with both: the clinical sections written professionally, and the instruction sections written so the patient can act on them.

Instruction as usually written	Instruction the patient can act on
‘Continue antibiotics’	‘Take one Amoxicillin 500 mg capsule three times a day, after food, for five more days. Finish the whole course even if you feel better.’
‘Dressing as advised’	‘Keep the wound dry for three days. Come to the dressing room on 18 March for a change of dressing. Do not remove the dressing yourself.’
‘Avoid strenuous activity’	‘Do not lift anything heavier than five kilograms, and do not drive, for two weeks. Walking is encouraged.’
‘Report if any problem’	‘Come back at once if the wound becomes red, hot or starts leaking; if you develop fever; or if the pain becomes worse rather than better.’
‘Review SOS’	‘Come to the surgical outpatient clinic on 25 March at 10 a.m. Bring this summary and your medicines.’

8.14 Readmission and what the summary should have prevented

An early readmission is worth reading backwards as a documentation audit. In a substantial proportion of cases the discharge summary contains the explanation: a medication that was stopped without the patient being told, a pending result nobody followed, a warning sign the patient was not given, or a follow-up appointment that was never made.

- Audit a sample of readmissions within seven days against the original discharge summary
- Check specifically for omitted or duplicated medication, pending investigations, absent warning signs and absent follow-up
- Feed the findings back to the discharging unit by document element, not as a general exhortation
- Track whether the proportion of readmissions with an identifiable documentation contributor falls

8.15 Death in specific circumstances

Circumstance	Additional documentation
Brought dead	Time of arrival and time of declaration, the examination performed, the history from whoever brought the patient, identification attempts, medico-legal declaration, and police intimation where the cause is unknown
Death in the emergency department	The resuscitation record with drugs, times and rhythms, the decision to stop with the time and who made it, and who was present
Maternal death	A full case review as the facility reporting system requires, the sequence of events with times, and notification under the maternal death surveillance arrangements
Neonatal and perinatal death	Birth record, gestational age, birth weight, APGAR scores, resuscitation record, and the separate perinatal death documentation
Death during or after a procedure	The operative and anaesthesia records complete, medico-legal referral, body preserved, and no certificate issued ahead of the statutory process
Death of an unidentified person	Identification attempts, photographs and identifying marks recorded, police intimation, and the statutory route for unclaimed bodies
Potential organ donation	Brain-stem death certification by the prescribed panel on the prescribed form, the statutory authorisation, and the donor documentation kept per the transplantation law

8.16 The death audit

Mortality review is a NABH expectation and it depends entirely on the record. A review that can only establish that the patient died has no value. A review that can trace the clinical trajectory, the decisions taken, the escalations made and the communication with the family can identify what, if anything, should change.

1. Review every death against a standard tool, with a defined sample reviewed in depth.
2. Examine the record for the trajectory: were deteriorations detected, escalated and responded to in time?
3. Examine documentation quality separately from clinical care, because a death may be unavoidable and the record still inadequate.
4. Classify deaths by preventability using a defined scale, with the reasoning recorded.
5. Record the actions arising and review them at the next meeting.
6. Keep mortality review minutes as a protected quality document in line with hospital policy, while ensuring the clinical record itself contains the clinical facts.

8.17 Leaving from the emergency department

Emergency departments have their own exit routes, and each needs documentation that the inpatient discharge process does not cover. These are among the most frequently litigated episodes in Indian hospitals, because the contact is short and the record is thin.

Route	What the record must show
Discharged home after treatment	Reassessment before discharge with the time, the diagnosis or working impression, treatment given, specific return advice, follow-up arranged, and the condition at departure
Left without being seen	Time of registration, triage category and time, the attempts made to call the patient, the time the absence was identified, and that the triage category was considered in deciding whether to follow up
Left against advice from casualty	Capacity, what was explained including the specific risk of leaving, alternatives offered, the signed form, the witness, and the return advice still given

Route	What the record must show
Referred or transferred out	The reason, the receiving facility and the doctor accepting, the condition at transfer, treatment given with times, and the mode of transport and escort
Admitted	The admission decision with the time, the accepting unit and consultant, the handover given, and the time the patient physically moved
Died in the department	The resuscitation record, the time and method of confirmation, the medico-legal screening decision, who was informed, and the body handover

> **CAUTION**
> A patient who leaves without being seen after a high-acuity triage category is a patient the hospital assessed as potentially seriously ill and then lost. The record must show what the department did about it; in most cases that means documented attempts to contact the patient.

8.18 Certificates patients ask for

Hospitals issue a steady stream of certificates, and each one is a document created by the hospital, carrying its name, that will be relied upon by somebody. They should be issued from the record, by a person entitled to issue them, and a copy should be retained.

Certificate	Issued on the basis of	Controls
Medical certificate for sick leave	The clinician's own assessment of the patient during the period certified	Dates stated precisely; never backdated beyond the period actually assessed; a copy retained in the record
Fitness to resume duty	An assessment at or near the date of the certificate	The assessment recorded in the notes; any restrictions stated
Discharge or treatment certificate for insurance	The record of the admission	Issued through the records department against a request; a copy retained; content consistent with the summary
Disability certificate	The statutory assessment process and the prescribed board	Issued only by the authorised board on the prescribed form; the assessment documented
Birth and death certificates	The institutional record and the statutory registration process	The hospital issues the record of the event; the registering authority issues the legal certificate; counterfoils retained permanently
Fitness for travel, sport or employment	An assessment appropriate to the purpose, which may be beyond the treating clinician's scope	Decline where the assessment required is outside the clinician's competence, and record the reason

Two rules carry most of the risk. A certificate is never issued for a period the clinician did not assess, and a certificate is never issued for a patient the clinician has not seen. Both requests are made constantly and both are refused in writing, with the refusal recorded.

8.19 Putting this module to work

1. Take one discharge summary from your ward and test it: could a doctor in another city manage this patient from it alone?
2. Check whether the 'return urgently if' section names specific signs or says 'report if any problem'.
3. Check whether any patient discharged last week had a pending investigation, and whether the summary names it and says who will follow it up.
4. Look at the last death certificate issued in your unit. Does Part I show a causal sequence ending in an underlying cause?

5. Check whether your hospital documents the medico-legal screening decision for every death, or only for the obvious cases.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A patient is discharged after a four-day admission for pneumonia. The summary lists the final diagnosis as 'fever for evaluation', which was the admitting provisional diagnosis. What are the consequences?**

Answer: Clinically, the next clinician does not know what was treated or whether it resolved. For coding, no principal diagnosis can be assigned, so the claim is likely to be rejected or down-coded. For the hospital record, the file shows four days of treatment for an undiagnosed condition. The diagnosis sheet and the summary must both be updated to the final diagnosis with its co-morbidities and complications, which is a clinical responsibility and not a coding one.

Q2. **A patient is transferred to a tertiary centre. The transfer note says 'Referred for higher management' and copies of investigations are not sent. What is missing and what is the clinical consequence?**

Answer: Missing: the reason for transfer, the accepting doctor's name and the time of acceptance, the condition and last observations at transfer, treatment given with last doses and times, lines and devices with insertion dates, investigations done with results, outstanding investigations, mode of transport and escort level, and what the family was told. Clinically, the receiving team restarts the workup, repeats investigations, delays definitive treatment, and may repeat an intervention that has already failed.

Q3. **A death certificate records Part I (a) as 'cardiorespiratory arrest' and nothing else. Why is this unacceptable and what should it say?**

Answer: Cardiorespiratory arrest is the mode of dying and is common to every death, so it conveys no information and cannot be coded. Part I must record the causal sequence: the immediate cause at (a), the condition giving rise to it at (b), and the underlying cause that started the sequence at the lowest line, which is the one coded for mortality statistics. For example: (a) septic shock, due to (b) perforated appendix, due to (c) acute appendicitis.

Q4. **A patient leaves the ward without telling anyone. The nurse writes 'Patient absconded' and completes a normal discharge summary. What should have happened?**

Answer: An absconding patient generates an incident record, not a planned discharge. The record must show the time the patient was last seen, the time the absence was noted, the search conducted, who was informed and when — relatives, consultant and administration, and the police where the patient is a minor, lacks capacity, is a medico-legal case or is at risk — together with any devices still in situ and the belongings left behind. Issuing a normal discharge summary misrepresents what happened.

Q5. **A patient dies two hours after returning from theatre following an elective procedure. The team completes a death certificate and releases the body to the family. What has gone wrong?**

Answer: An unexpected death during or shortly after a procedure or anaesthesia requires medico-legal referral. The hospital should have documented the screening decision, informed the police, preserved the body and any material evidence, and not issued a certificate ahead of the statutory process. Releasing the body has removed the possibility of a post-mortem examination, which damages the family's position as well as the hospital's.

Q6. **A chart carries the letters 'DNR' in the corner, initialled, with nothing in the notes. The patient arrests and is not resuscitated. The family alleges they were never consulted. What is the hospital's position?**

Answer: Indefensible. A decision to limit treatment requires the clinical basis, the discussion with the patient where possible and with the family, any advance directive, the identity of those participating in the decision, the precise scope of what is and is not to be done, and the plan for symptom control. Three initialled letters evidence none of this, and the hospital cannot show that the decision was taken properly, let alone that the family was involved.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.